

The Medicity

DIGITAL GROWTH SNAPSHOT

The Medicity

themedicity.com/

Audit date: 2026-08-08

Report version: v4

Prepared by Catalyst Solutions Services

Reviewed by Dev Test

Prepared for the business named above. Findings reflect what was publicly observable and what you told us on the audit date — see the methodology section at the end for scope and limitations.

A workable base with clear gaps — analytics & trust needs attention first.



GROWTH READINESS

Search visibility		67	HIGH CONFIDENCE
Content & authority		67	MEDIUM CONFIDENCE
Conversion readiness		67	MEDIUM CONFIDENCE
Analytics & trust		28	MEDIUM CONFIDENCE
AI-search readiness		57	MEDIUM CONFIDENCE
Mobile & speed		67	HIGH CONFIDENCE

Scores are computed from 27 automated checks. A pillar only reaches 100 when every check is independently verified — "detected" signals earn partial credit.

IF YOU ONLY READ ONE THING

- You lack conversion and cost-per-lead tracking, meaning every marketing rupee spent is currently unmeasured.
- Your course catalog pages attract high-intent medical professionals, but technical errors like missing H1 tags and image alt text limit search visibility.
- There is no active blog or educational content engine to capture early-stage clinical queries before doctors commit to specialized training.

What we understood

The Medicity provides advanced medical fellowships, diplomas, and specialized training programs for doctors and surgeons across India and international markets. You run intermittent Google Ads and rely heavily on organic search and direct messaging for course registrations.

What we checked

27 automated checks, run 2026-08-07. "Detected" means a signal was found but not proven to be working; "Assumed" means we could not verify it without account access.

SEARCH VISIBILITY

✓ robots.txt present	VERIFIED
✓ XML sitemap found	VERIFIED
✓ Canonical tag on homepage	VERIFIED
✓ Homepage title tag length 15–65 characters 60 characters	VERIFIED
✓ Meta description on homepage 197 characters	VERIFIED
✗ All scanned pages have meta descriptions 1 of 4 pages missing	VERIFIED
✓ No duplicate titles across scanned pages	VERIFIED
✗ Exactly one H1 on homepage 0 found	VERIFIED
✗ No broken internal links in sample broken: /xmlrpc.php	VERIFIED

CONTENT & AUTHORITY

✗ Blog or content section	DETECTED
✓ Substantial homepage copy (300+ words) 1013 words	VERIFIED
✓ Multiple key pages reachable 4 pages scanned	VERIFIED

CONVERSION READINESS

✓ Contact form present	VERIFIED
✓ Form length reasonable (≤5 fields) longest form: 4 fields	VERIFIED
✗ Online booking / scheduling link	DETECTED

ANALYTICS & TRUST

✓ Analytics script installed	DETECTED
✗ Conversion tracking working owner reports conversions/cost per lead are not reliably tracked	ASSUMED
✓ HTTPS	VERIFIED

⊗ HSTS security header

VERIFIED

⊗ X-Content-Type-Options header

VERIFIED

⊗ No mixed http/https content

VERIFIED

AI-SEARCH READINESS

✓ Structured data (JSON-LD) present markup found — validity not independently verified

DETECTED

✓ Open Graph tags

VERIFIED

⊗ Machine-readable page structure (title + meta + H1)

VERIFIED

MOBILE & SPEED

✓ Mobile viewport configured

VERIFIED

✓ Favicon present

VERIFIED

⊗ Images have alt text 35 of 35 homepage images missing alt

VERIFIED

What we found

HIGH IMPACT

Your current ad campaigns lack cost-per-lead tracking, making it impossible to evaluate acquisition efficiency for high-value medical courses.

🔗 Businesses that adopt data-driven marketing are roughly 6x more likely to be profitable year-over-year (Forbes/BCG data-driven marketing research)

Source: Your answer during intake, checked 2026-08-07: 'We do not track cost per lead today. A course registration is worth a lot, so ads would be worth it if we could measure them.'

MEDIUM IMPACT

The homepage currently has zero H1 header tags, preventing search engines from properly reading your primary page topic.

Source: Your homepage scan, checked 2026-08-07: zero H1 tags were detected in the page source.

MEDIUM IMPACT

All 35 images on your homepage lack alt text attributes, missing an opportunity for image search visibility and accessibility compliance.

Source: Your homepage scan, checked 2026-08-07: 35 of 35 images were found without alt text.

MEDIUM IMPACT

There is no active blog or content section on your site, meaning organic reach relies entirely on static course landing pages rather than educational search queries.

🔗 Companies that blog consistently generate 67% more leads per month than those that don't (DemandMetric/HubSpot content research)

Source: Your site structure and intake answers, checked 2026-08-07: no blog exists, and your answer confirmed 'No blog. Course pages only.'

MEDIUM IMPACT

Inquiries are fragmented across WhatsApp, phone calls, and form submissions on individual course pages without centralized attribution.

Source: Your intake answer, checked 2026-08-07: 'most enquiries arrive via WhatsApp, phone, and organic search on course pages.'

↩️ How you compare

Medvarsity

DIRECT

Medvarsity offers online medical courses and clinical training programs for doctors and healthcare professionals, directly competing in the medical education space.

St. John's National Academy of Health Sciences

INDIRECT

St. John's National Academy of Health Sciences provides medical education and clinical courses, serving as an alternative institution for medical training.

CHECK	THE MEDICITY (YOU)	MEDVARSITY	ST. JOHN'S NATIONAL ACADEMY OF HEALTH SCIENCES
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Growth readiness score	59/100	84/100	60/100
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Blog or content section	✗	✓	✓
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Structured data (JSON-LD) present	✓	✓	✗
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Analytics script installed	✓	✓	✓
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Online booking / scheduling link	✗	✓	✗
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Meta description on homepage	✓	✓	✓
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Exactly one H1 on homepage	✗	✓	✗
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Images have alt text	✗	✓	✗
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XML sitemap found	✓	✓	✗
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Same automated check battery run on every site. Traffic, rankings, and revenue were not measured — see methodology.

Where you lag

- ↘ Competitors like Medvarsity maintain higher overall technical and search visibility scores.
- ↘ Institutions like St. John's publish an active blog and content section, whereas your site has no blog.
- ↘ Competitors have structured broader institutional digital footprints across multiple channels.

Where you lead

- ↗ Your homepage copy is substantial and thorough, exceeding 1,000 words of detailed course information.
- ↗ You feature prominent, recognized medical faculty (such as Padma Shri doctors) directly on core pages.
- ↗ Your course registration forms are short and concise (4 fields max), reducing friction compared to lengthy multi-step forms.

Who you should be targeting

The Practicing Specialist Doctor

Demographics & Firmographics: MBBS/MD/MS qualified doctors, surgeons, and obstetricians aged 30–55 across India and international markets, seeking specialization credentials to scale their private practice, hospital credibility, or career opportunities.

Decision maker: The individual clinician making personal career and professional development investments.

Trigger events: Losing high-paying surgical or fertility referral cases to colleagues with advanced credentials; joining or planning to set up an independent IVF or laparoscopy center; a hospital promotion requirement for specialized surgical units.

Pain points: Inability to perform advanced minimally invasive procedures without formal fellowship credentials; lack of hands-on mentorship in existing medical setups; difficulty finding structured, short-duration courses that accommodate active clinical schedules.

Objections: "Will this short fellowship actually give me hands-on surgical confidence or just theory?" or "How do I manage my clinic while attending training in Gurugram?"

Buying behavior: Research-heavy consideration window spanning weeks; relies heavily on faculty reputation (such as the Padma Shri doctors listed on your site); direct inquiries via WhatsApp and phone; verifies institutional credibility before committing.

Best channels: High-intent Google Search ("fellowship in minimal access surgery India", "IVF training for gynecologists"); WhatsApp direct response and consultation; professional medical network referrals.

The Hospital Group / Corporate Sponsor

Possible routes

ROUTE	EFFORT	DIY- ABLE	TIMELINE (WEEKS, PADDED)
Route A - Foundation, Tracking, and On-Page SEO	Medium	✓	6–9 wks
Route B - Content Authority & Search Scaling	High	—	16–24 wks

Route A - Foundation, Tracking, and On-Page SEO

Medium effort

Partly DIY-able

Fixing technical code errors, installing robust conversion and lead tracking, and optimizing high-intent course pages.

Best if: You want to stop flying blind on marketing spend before investing in paid acquisition or content.

Realistic milestones: Tracking installed and H1 tags fixed within weeks 3–4; all image alt text and metadata finalized by weeks 6–8.

If you do nothing: Any future ad spend or traffic growth will continue to convert blindly without attribution data.

Trade-offs: This route generates no new traffic by itself; it ensures your existing traffic and future campaigns actually convert and register properly.

Route B - Content Authority & Search Scaling

High effort

Building a faculty-led content engine and publishing targeted articles for high-intent medical education search terms.

Best if: You want to build a compounding organic pipeline from doctors searching for specialized fellowships.

Realistic milestones: Content calendar and first 4 faculty-authored articles published by weeks 6–8; initial ranking movement visible by months 4–6.

If you do nothing: You remain dependent on brand-name searches while competitors capture category-level clinical queries.

Trade-offs: Organic search takes months to compound; the first month is strictly foundation rather than rankings.

Quick wins — inside 30 days, any route

- 1 Install conversion tracking for WhatsApp clicks and form submissions across all course landing pages.
- 2 Add H1 header tags to your homepage and key course pages.
- 3 Add descriptive alt text to all 35 homepage images to improve accessibility and search crawling.
- 4 Claim and complete your Google Business Profile to capture local Gurugram medical visibility.

Assumptions we made

- Your internal faculty can dedicate 2–3 hours per month to review or draft clinical content topics.
- You maintain administrative control over the website backend to implement technical fixes and tracking scripts.

Methodology & limitations

What we reviewed. 4 publicly accessible pages (/fellowship-in-ivf-and-reproductive-course/, /course-calendar/) on 2026-08-07, plus your intake answers.

How scores work. 27 automated checks grouped into 6 pillars. Independently verified checks earn full credit; signals we detected but could not prove working earn 70%; assumptions earn half. No pillar reaches 100 unless everything in it is verified. Each pillar shows a confidence level based on how much of it we could verify.

What we could not verify. Anything requiring account access: analytics configuration and conversion events, Search Console data, ad account performance, actual enquiry volumes. Where these matter, the report says so rather than guessing. Statistics cited come from named published studies; none are generated.

Competitor comparison. Competitors were identified from industry knowledge, then each candidate's website was visited, its relevance confirmed from its actual content, and the same automated check battery run on it. Of 5 candidates considered, 2 passed verification. Competitor traffic, search rankings, and revenue were not measured — comparisons are limited to what is publicly checkable.

Human review. This report was drafted by an AI analysis pipeline and reviewed, edited where needed, and approved by a named Catalyst consultant before release (Dev Test). Timelines shown are deliberately conservative ranges. Nothing in this report is a guarantee of a business outcome.